



Occupational Therapy Home Assessments Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	Allied Health	Occupational Therapy (OT)

2. Purpose

To ensure the Occupational Therapy home assessment service maintains staff safety and provides equitable patient access, consistent assessment, intervention, and discharge processes. This procedure is to be read in conjunction with the [EMHS Community/home visiting Policy](#).

Occupational Therapy home assessments are provided to eligible AKG patients to assess in home function, prescribe environmental adaptations and provide education to maximise patient safety and function within their home environment. Occupational Therapy Assistants (OTA) and Allied Health Assistants (AHA) conduct home visits for the delivery and pick-up of equipment and the Occupational Therapy Home Care Carpenter conducts visits to install prescribed installations and fabrications.

3. Procedure

3.1 Car Booking

3.1.1 Armada

- OT van can be booked via the diary under the sign-in book in OT outpatient area next to sign in book.
- Pool vehicle bookings are via the online booking system under public folders in Outlook (Public folders/Armada Health Service/Resources/Car bookings.)

3.1.2 Kalamunda

- Send an Outlook meeting request to KDCH.Vehicle@health.wa.gov.au.

3.1.3 Both Campuses – Own Vehicle Use

- Staff members are **not** to drive patients in their own vehicles regardless of insurance cover.
- Use of private vehicles for home visiting is not encouraged and private vehicles are only be used where a hospital vehicle is unavailable or when an assessment is on the way to or from work.
- Most car insurance policies do not cover the use of private vehicles for business use.
- Employees choosing to use their own vehicle are responsible for checking and ensuring they have adequate insurance coverage.
- Staff members with a medical/disability requirement to use their own vehicle, require authorisation from the Head of Occupational Therapy and are responsible for ensuring their vehicle is insured for business use.

3.2 Environmental factors

- Home visits are not undertaken to areas with current fire risks. Check fires/fire risk via The [Department of Fire and Emergency Services](#). All pool cars should contain a fire safety kit.
- Storm risk can be checked via the Bureau of Meteorology.
- Home visits are not undertaken to areas where there is a current safety risk e.g., gunman in the area. Check [WA national news](#) for any news alerts.
- If unsure if visit should proceed, discuss with line manager or Head of Department.

3.3 Risk Assessment and Safety

- Complete the pre home visit risk assessment as per [EMHS Community/home visiting Policy](#). Discuss any safety concerns with the line manager or head of department. If a visit is deemed unsafe service can be declined. Alternatively, an action plan can be formulated which may include a two person visit and/or phone call checks.
- A Home Visiting Schedule Form is to be fully completed for all visits and left with the Community Rehabilitation Reception staff, ward clerks for Rehab OTs, or another OT if outside reception hours or the front reception desk at Kalamunda. See Appendix 1 which has a risk assessment check list on the back of the form.
- When conducting home visits on the way to work, the OT leaves the home visit schedule form with an appropriate person the day before and informs their line manager of the expected arrival time. Write in the sign-in book when you are expected at work.
- When completing home visits on the way home the Home Visiting Schedule Form is left with an appropriate person and phone contact needs to be made when the staff member leaves the visit.
- OTs, OTAs, TAs and carpenter are to take an Ottag personal alarm with them on all home visits. This is a monitored alarm and will let the HOD, Line Mx and Police know if the alarm has been activated. If it has been activated accidentally then you will need to state your HE number and say stand down. These are kept on charge in the:
 - Disability Equipment and Home Modifications office for the carpenter to pick up each morning and DEHM OT and Deliveries as required. Please place back on charge when you return from your home assessment.
 - Community Rehab (CR) for the OTs undertaking Home Ax, Falls Specialist Team, OLS OT/SW, and Therapy Assistants undertaking home therapy programs. Alarms should be on charge when not in use. Please place back on charge when you return from your home assessment.
 - Rehab Ward Seniors OTs office for OTs undertaking Home Axs. Please sign them in and out when using them and return to charge.
 - Kalamunda Hospital – for OTs undertaking Home Axs. Please sign it in and out when using them and return to charge.

These work anywhere that there is Telstra phone reception, so there will still be areas where there is no reception, and it is important to take your phone and complete your risk Ax form to provide for your best safety.

3.4 Phones

- An adequately charged mobile phone is to be always kept on the OT staff member during home visits. AKG OT mobile phones are kept in the OT Department, and CSRU ward OT offices at Armadale and at front reception at Kalamunda.
- If delayed the staff member will notify the person holding the home visiting schedule form of the new expected return time. Where delay is past reception staff finishing time the schedule form

will be given to one of the Senior Occupational Therapists. The Senior OT will ensure or assign another person to ensure the staff member returns to base at the anticipated time.

- If the staff member has not made contact or returned by the assigned time the person holding the home visit schedule form will attempt to make phone contact with the therapist. If unable to make contact a Senior OT or head of department will be notified to decide if further action is required.

3.5 Training

- All staff members must attend Aggression Prevention Intervention at Armadale Kalamunda Group at the first available course.
- All staff members conducting home assessments are to complete the Armadale Kalamunda Group Basic CPR training annually at mandatory training. Staff members are also encouraged to undergo additional first aid training.

3.6 Home Assessment with Current Inpatients

- Consult the patient's doctor to ensure the patient is medically stable enough for the visit.
- Plan visit time to avoid clashing with any essential medical procedures.
- Record the planned visit time in the patient's medical file and on whiteboard on the ward if available.
- Inform relevant team members of when the home visit is being conducted.
- Inform the nurse assigned to the patient when they leave the ward and when they are due to return – nurses can be contacted via the patient's bedside staff assist call bell.
- Park the car at the closest entry to the ward - ward wheelchairs can be used to transport the patient to the car if required.
- If transporting a patient on oxygen:
- Ensures that the patient has a full oxygen bottle prior to leaving on the visit.
- Use only a vehicle with a metal cargo barrier - the oxygen bottle is to be placed behind the cargo barrier and secured to the barrier; the cord can then be run through the car to the patient sitting in the front seat. Currently Car 24 Fleet Vehicle has a cargo barrier but check with Facilities Management (formally Engineering) as the fleet vehicles are upgraded and this may change.

3.7 Access

3.7.1 Disability Equipment and Home Modification Service (DEMH)

- Community Aids and Equipment Program (CAEP) clients who are over 65 and on the CAEP data base are eligible for ongoing care under this program.
- Clients under 65 on the CAEP data base but have not yet transitioned to NDIS will also be seen by this service until they have an active NDIS plan.
- Clients under 65 who have been newly diagnosed with a disability e.g. MND, MS, can be considered by CAEP while they wait for an NDIS plan on a case-by-case basis. Also, clients under 65 who are not eligible for NDIS with permanent medical conditions e.g. COPD, Cardiac conditions can be considered for CAEP funding for equipment. An application is made to East Metro CAEP Co-ordinator for approval to proceed with intervention.
- Clients who have used all their My Aged Care package funding for services and need specialised equipment that cannot be hired, can also be considered on a case-by-case basis with proof that package finances have been spent
- General Practitioner referrals for clients under 65 that do not meet NDIS criteria but are at risk of inpatient admission without intervention will be seen for basic and essential intervention.

- Client's undergoing oncology treatment who requires temporary or short-term assistance will also be seen under this program.
- Any patients that present at the Armadale Health Service Emergency Department and typically live in the Armadale, Kalamunda and Bentley catchment areas and are identified as requiring an access and/or functional home visit can be referred to DEHM via **eReferral**.
- All patients who are deemed clinically safe to return home, prior to an OT home visit are referred by the ward therapist to this service via **eReferral** for a community home visit.

3.7.2 Inpatients

- Typically, those patients who require an access and /or functional home visit prior to discharge; client's out of catchment will also be included if clinically required. These patients may be seen by the ward therapist or DEHM depending on availability and waitlists.

3.7.3 Pre-Admission Clinic – booked hip and knee surgeries.

- An initial phone interview will indicate if the patient within the Armadale Kalamunda Group catchment area requires a preadmission home visit. If a home assessment is indicated, then it will be undertaken prior to admission if possible. Provide PAC kit to DEHM team with home assessment information to be completed. An eReferral is not required.
- For patients outside the Armadale Kalamunda Group catchment area, the preadmission occupational therapist will liaise with occupational therapist from the hospital that is closest to the patient to complete the home visit. If an occupational therapist home visit is unable to be completed, the pre-admission occupational therapist will send the Home Environment Measurements form for the patient to complete and return when the patient comes in for surgery.
- If the patient is out of catchment and considered a high risk on discharge due to their home environment, discuss with the Senior OT in DEHM for capability to assist with a home assessment.

3.7.4 Community Rehabilitation

- Patients receiving rehabilitation through Community Rehab are eligible for OT Home Assessment as part of their treatment program.
- Patients who are under 65 are eligible for OT Home assessment and funding of basic and essential equipment and environmental adaptations if they are not accessing NDIS at the time of the assessment.
- All patients over 65 are eligible if it is related to their current goals, otherwise referral to My Aged Care should be completed for home modifications and equipment.

3.8 Prioritisation

3.8.1 Disability Equipment and Home Modifications

- Patients are prioritised as per [Allied Health Provision of Aids/Equipment and Home Modifications Procedure AKG-PRO-0028-17](#).
- Prioritisation will include balancing home assessment caseload with the rest of the caseload.

3.8.2 Preadmission Surgery

- Patients are prioritised by date of booked surgery.

3.8.3 Inpatient Service

- Patients are prioritised in relation to expected discharge dates.

3.9 Assessment and Discharge Process

- Home assessment is completed as per the Occupational Therapy Home Assessment (AKMR 61.3.1) or relevant assessment form (for Falls Specialist Program), including formulation of an intervention action list.
- The Occupational Therapy Home Assessment form (AKMR 61.3.1) or relevant form (for Falls Specialist Program) will be completed within 2 days of the home environment assessment and scanned into the Digital Medical Records (DMR) with the patient sticker from the date of the visit attached to all pages.
- The occupational therapist will document any results from other assessments undertaken on the home assessment e.g., cognitive assessments, pressure risk assessments in the appropriate format in the DMR on the day of the home assessment.
- The assessing Occupational therapists will action items on intervention action list for which they are responsible. Note some items on action list will be a recording of recommendations for patient to action.
- Aids and equipment, home installations/modifications recommended on home assessment are issued or actioned as per the [Allied Health Provision of Aids/Equipment and Home Modifications Procedure AKG-PRO-0028-17](#) and the [Occupational Therapy Home Installations, Fabrications and Modifications Guidelines ALL-GUI-0322-17](#).
- Follow-up phone calls or home visits will be completed as clinically indicated. Follow-up is recorded in the DMR,
- The occupational therapist will recommend evaluation by other health professionals where further consultation would be beneficial. If the patient agrees the appropriate eReferral(s) will be actioned as required. Occupational therapist will document all referrals in the patient's medical file.
- Home assessment results will be communicated to relevant team members and/or referrer as required or requested.
- On completion of DEHM home assessment and follow up and a brief summary will be recorded in Clinical Manager as handover to inpatient staff who initiated the referral and/or for future admissions.
- Written feedback is provided to all external referrers when patients are discharged from the occupational therapy service e.g., General Practitioners, and a copy with date specific patient stickers attached on the front and back left in the scanning tray for DMR.

3.10 Waitlist Management

- DEHM and Community Rehabilitation – waitlists are reviewed and reprioritised regularly and balanced in conjunction with other caseload requirements.
- Inpatient - (General Medical Wards, Kalamunda Palliative Care and Allied Health ED Team) the ward/ED therapist will identify if a patient requires a home assessment and complete an eReferral to DEMH service using the template on the eReferral which includes a risk assessment, estimated discharge date for the patient, and priority recommendation. Feedback to the referring therapist is via clinical manager, in the relevant inpatient section of the digital medical record and verbally/email if required.

4. Roles and responsibilities

Role	Responsibility
Senior OT Disability Equipment & Home Modifications	Education to all new OT staff members during orientation on safety when home visiting. Regular updates in OT staff meetings. Updating policy as required maintaining currency and accuracy.
All OT Department Staff Members	To have read this policy as per the orientation checklist for their clinical area. To undertake the competency education and assessment during orientation. Certificate to be sent to HOD OT for Competency Records.

5. Legislative context

The following documents are required to give effect to this procedure:

- *Occupational Health, Safety and Welfare Act 1984*
- Department of Health Western Australian (HDWA) [Provision of Assistive Technology and Home Modifications Policy MP 0168/21](#)
- East Metropolitan Health Service [Community /Home Visiting Policy EMHSL 131](#)
- EMHS [Vehicles Policy EMHS: 134](#)

6. Supporting documents

- [Allied Health Provision of Aids/Equipment and Home Modifications Procedure AKG-PRO-0028-17](#)
- [Occupational Therapy Home Installations, Fabrications and Modifications Guidelines ALL-GUI-0322-17](#)

7. Compliance, monitoring and evaluation

HOD OT will maintain a current list of OTs who have completed the Home Visiting competency. Aim is 100%.

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Action 1.30 Safe environment
- Standard 6 – Action 6.3 Partnering with consumers
- Standard 6 – Action 6.4 Organisational processes to support effective communication
- Standard 6 – Action 6.11 Documentation of information

7. Policy owner

Head of Department Occupational Therapy

Enquiries relating to this procedure may be directed to:

Title: Senior Occupational Therapist Disability Equipment and Home Modifications
Department: Occupational Therapy

9. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
Version 3.1	13/11/2024	13/04/2027	Additional that use of private vehicles is not recommended. Additional that private car insurance generally does not cover business use and employee responsibility to check this.

10. Authorisation

This procedure has been authorised and issued by the Director Allied Health.

Authorised by	Director Allied Health & Ambulatory Care
Authorisation date	13/11/2024
Published date	13/11/2024

Appendix 1: Home Visiting Schedule - Front Page

REHABILITATION AND AGED CARE SERVICE OCCUPATIONAL THERAPY HOME VISITING SCHEDULE

NAME: _____

DAY/DATE: _____

CAR NUMBER: _____ MOBILE PHONE NUMBER: _____

TIME OUT: _____ ESTIMATED TIME BACK: _____

DETAILS OF VISITS: (include name and telephone number of patients to be visited, in order.
Please add any other information which may be useful.)

REHABILITATION AND AGED CARE SERVICE OCCUPATIONAL THERAPY HOME VISITING SCHEDULE

NAME: _____

DAY/DATE: _____

CAR NUMBER: _____ MOBILE PHONE NUMBER: _____

TIME OUT: _____ ESTIMATED TIME BACK: _____

DETAILS OF VISITS: (include name and telephone number of patients to be visited, in order.
Please add any other information which may be useful.)

Appendix 1: Home Visiting Schedule - Back Page**PRIOR TO VISIT/ADMISSION ASSESSMENT/PRE-VISIT ASSESSMENT**

- Location of Home – isolated, not on map, need 4WD
- Who will be present at the visit?
- Attitude of the family/spouse? Supportive/Not
- Any Disputes with neighbours? Y/N
- Do they smoke? Y/N If yes, can you refrain from smoking during the visit?
- Do you have a dog? Y/N If yes, can it be locked outside during the visit?
- Do you currently have a contagious illness e.g. measles, or flu. Y/N
- Are you coughing/sneezing? Y/N If so, would you be prepared to wear a mask during the home assessment to minimise risk to the therapist/carpenter? Y/N
- Known drug/alcohol use – ascertain from MR
- Alerts on Medical Records e.g. history of violence – ascertain from Webpas/MR/eReferral Risk Ax.
- Fill out Home Visiting Schedule form and leave with the receptionist on your work area or Senior OT.
- Ensure you have a mobile phone and Otag to take with you from the Senior DEHM office. The phone has ICE, Police numbers and your senior/principal's numbers programmed into it and the Otag, once activated, will let senior OT staff & police know of incident.

PRIOR TO VISIT/ADMISSION ASSESSMENT/PRE-VISIT ASSESSMENT

- Location of Home – isolated, not on map, need 4WD
- Who will be present at the visit?
- Attitude of the family/spouse? Supportive/Not
- Any Disputes with neighbours? Y/N
- Do they smoke? Y/N If yes, can you refrain from smoking during the visit?
- Do you have a dog? Y/N If yes, can it be locked outside during the visit?
- Do you currently have a contagious illness e.g. measles, or flu. Y/N
- Are you coughing/sneezing? Y/N If so would you be prepared to wear a mask during the home assessment to minimise risk to the therapist/carpenter? Y/N
- Known drug/alcohol use – ascertain from MR
- Alerts on Medical Records e.g. history of violence – ascertain from Webpas/MR/eReferral Risk Ax.
- Fill out Home Visiting Schedule form and leave with the receptionist on your work area.
- Ensure you have a mobile phone and Otag to take with you from the Senior DEHM office. The phone has ICE, Police numbers and your senior/principal's numbers programmed into it and the Otag, once activated, will let senior OT staff & police know of incident.